**Which treatment this guide covers** NCCP 00220a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is continuous treatment for advanced disease, not a fixed adjuvant course.

NCCP 00220 · VERSION 5

# Gefitinib monotherapy

The usual dose is 250 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

BASELINE → TREATMENT → FOLLOW-UP

## Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

| Check | Before the first dose | Early treatment review | Ongoing daily treatment | Benefit / toxicity review | Treatment change or completion |
|---|---|---|---|---|---|
| **Treatment** | Confirm indication and prescribed dose | Daily gefitinib; no planned rest week | Continue the prescribed dose | Review benefit, harms and dose | Agree next treatment or follow-up |
| **Blood tests** | Blood count, kidney and liver profile | Repeat as clinically indicated | Team sets follow-up tests | Extra tests for dehydration or liver problems | Follow unresolved abnormalities |
| **Symptoms and medicines** | Review lungs, bowel, skin, eyes and medicines | Report bowel, skin or eye symptoms early | Check adherence, supply and interactions | Confirm recovery before restart | Continue advice for unresolved symptoms |
| **Cancer assessment** | Confirm EGFR, stage and prior treatment; chest X-ray and CT | Clinical review | Scans per Consultant plan | Discuss response or progression | Agree follow-up or next treatment |

THE WHOLE TREATMENT JOURNEY

# Gefitinib monotherapy: treatment and checks

## Activating-EGFR advanced NSCLC

The usual dose is 250 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before the first dose | Early treatment review | Ongoing daily treatment | Benefit / toxicity review | Treatment change or completion |
|---|---|---|---|---|---|
| **Treatment** NCCP 00220 · SMPC | Confirm indication and prescribed dose | Daily gefitinib; no planned rest week | Continue the prescribed dose | Review benefit, harms and dose | Agree next treatment or follow-up |
| **Blood tests** NCCP 00220 · SMPC | Blood count, kidney and liver profile | Repeat as clinically indicated | Team sets follow-up tests | Extra tests for dehydration or liver problems | Follow unresolved abnormalities |
| **Symptoms and medicines** NCCP 00220 · SMPC | Review lungs, bowel, skin, eyes and medicines | Report bowel, skin or eye symptoms early | Check adherence, supply and interactions | Confirm recovery before restart | Continue advice for unresolved symptoms |
| **Cancer assessment** NCCP 00220 · SMPC | Confirm EGFR, stage and prior treatment; chest X-ray and CT | Clinical review | Scans per Consultant plan | Discuss response or progression | Agree follow-up or next treatment |

# What each check is for

### 1 Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

**Ask / act:** Document exact indication, dose and initial investigations.

Sources: NCCP 00220

### 2 Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

**Ask / act:** Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

Sources: NCCP 00220 · SMPC

### 3 New respiratory symptoms

At any time

Possible ILD requires urgent investigation.

**Ask / act:** Arrange urgent assessment and an authorised medicine plan.

Sources: NCCP 00220 · SMPC

### 4 Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

**Ask / act:** Agree continuation, hold, restart, change or follow-up.

Sources: NCCP 00220

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.26 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending